BOARD OF ADVISORS · INAUGURAL MEETING

Plexus AI

Advisory Board Agenda

A structured discussion guide for the inaugural advisory session — covering governance, clinical strategy, validation frameworks, business model, and the decisions required for execution.

SESSION TYPE	DATE
Full Board · In Person	April 2026
TOTAL DURATION	**CLASSIFICATION**
~110 Minutes	Confidential

SESSION AGENDA

7 agenda items · **~110 minutes total**

01	**Executive Overview & Mission Clarity** Problem, positioning, and Plexus AI's unique role	15 min
02	**Model Design & Governance Deep Dive** Operating model, governance structure, board oversight	20 min
03	**Clinical Integration & Adoption** Doctor buy-in, rollout strategy, specialty prioritisation	15 min
04	**Validation Framework & Evidence Generation** RWE criteria, methodology, publication strategy	15 min
05	**Business Model & Partnerships** Revenue streams, partner prioritisation, market strategy	20 min
06	**Scalability & Multi-Centre Strategy** Expansion phases, replicability challenges, funding for growth	15 min
07	**Key Decisions & Next Steps** Critical path decisions, action items, next board meeting	10 min

01 Executive Overview & Mission Clarity

⏱ 15 minutes

PROBLEM STATEMENT

Why Plexus AI exists

- → Healthcare AI achieves impressive lab results — real-world clinical translation rate remains **<10%**
- → Core bottleneck: gap between innovation and structured clinical validation
- → Root causes: absence of clinician involvement, poor workflow integration, lack of structured RWE

UNIQUE POSITIONING

What makes Plexus AI different

- → Hospital-embedded CoE — not a consulting firm, not pure tech, not a hospital innovation cell
- → Active clinician-led validation using real patient data
- → Structured pathway: Pilot → Validation → Scale
- → Real-World Evidence (RWE) as the primary, monetisable deliverable

BOARD DISCUSSION

- ? What is our primary success metric at 12 months — and does the board agree on the definition?
- ? How do we define "validation" in a way that satisfies regulators, clinicians, and investors simultaneously?
- ? What is the minimum viable proof point to attract our first paying partner?

02 Model Design & Governance Deep Dive

⏱ 20 minutes

CORE OPERATING MODEL

Five activities that drive the CoE

- → **Partner Onboarding:** Structured intake, scope definition, and expectations alignment
- → **Clinical Deployment:** Integration with OPD, IPD, ICU, and diagnostic workflows
- → **Doctor Engagement:** Structured feedback loops, usability testing, clinician training
- → **Data Capture:** Real-time usage, clinical outcomes, and efficiency metrics
- → **Validation & Reporting:** RWE generation, case studies, and publishable results

BOARD DISCUSSION

- ? Should Plexus AI establish a formal steering committee from day one? Who should sit on it?
- ? How do we balance clinical independence with partner commercial interests?
- ? What regulatory or ethical certifications should Plexus AI pursue early?

Key tension: Maintaining scientific independence while generating revenue from the partners whose tools are being validated. A clear governance charter is critical from day one.

03 Clinical Integration & Doctor Adoption

15 minutes

THE REAL CHALLENGE

Doctor buy-in

- → Doctors don't adopt tools that add friction — success means **reduced workload + visible benefit**
- → Integration into existing workflows (EHR, diagnostics, ward rounds) is non-negotiable
- → **Pilot bias:** Early adopters are not representative of broader clinical adoption potential

PHASED CLINICAL ROLLOUT

PRE-PILOT **Weeks 1–4**	Workflow mapping · clinician onboarding · baseline data collection
PILOT **Weeks 5–12**	30-day limited deployment · structured feedback · interim analysis
FULL DEPLOY **Month 4+**	Expanded access · continuous RWE capture · workflow optimisation

BOARD DISCUSSION

- ? Which clinical specialties should be priority #1? (Oncology? Emergency? ICU? Diagnostics?)
- ? Should Plexus AI hire dedicated clinical coordinators or rely on hospital staff?
- ? What incentive structures will sustain doctor engagement beyond initial pilots?

Clinician adoption rate is the single most powerful validation signal. A tool used by 80% of doctors in a pilot is worth more commercially than perfect outcomes in 20% of cases. Adoption rate should be a headline KPI.

04 Validation Framework & Real-World Evidence

15 minutes

WHAT MAKES RWE CREDIBLE

- → **Clinical rigour:** Defined outcomes, control or comparison groups where possible
- → **Transparent reporting:** Methodology, limitations, and conflicts of interest declared
- → **Generalisability:** Data representative of real populations, not just pilot cohorts
- → **Independent validation:** Third-party review or peer-reviewed publication

VALIDATION TIMELINE

MONTHS 1–3	Baseline data collection · outcome metric definition · initial feedback loops
MONTHS 4–6	Interim analysis · statistical review · publication preparation
MONTHS 9–12	Final RWE report · case study published · regulatory dossier compiled

PROPOSED RWE FRAMEWORK

01	02	03	04
Define	**Deploy**	**Analyse**	**Report**
Outcome metrics, population, comparison group	Clinical integration, data capture infrastructure	Interim and final statistical analysis	Case study, publication, regulatory dossier

BOARD DISCUSSION

- ? Should RWE be Plexus AI's primary revenue driver or a partnership enabler?
- ? Who owns publications? What journal impact threshold is required?
- ? Should validation be third-party audited? What certification model?

05 Business Model & Partnerships

⏱ 20 minutes

REVENUE STREAMS

- → **Validation Programs:** Structured per-engagement fee from AI and MedTech startups for end-to-end clinical validation
- → **Sponsored Research:** Pharma and MedTech companies commission independent RWE studies through the embedded network
- → **Grants & Partnerships:** DST, BIRAC, and ICMR funding for nationally-aligned innovation programs

PARTNER PRIORITISATION

TIER 1 · PRIORITY
Diagnostic AI
High impact · fast validation cycle · proven clinical demand

TIER 2 · NEAR-TERM
Clinical Workflow AI
EHR, notes, triage — high clinician touchpoint frequency

TIER 3 · PIPELINE
MedTech Devices
Longer cycles, higher revenue per engagement

TIER 4 · STRATEGIC
Pharma / RWE
High-value, relationship-driven, longer sales cycle

BOARD DISCUSSION

- ? Which revenue model is most sustainable — upfront fees, outcomes-based, or hybrid?
- ? What is the realistic total addressable market in India and globally?
- ? Should Plexus AI actively sell or build for inbound demand first?
- ? How do we prevent conflicts of interest when validating competing products?

Key tension: More rigorous validation = longer engagement = higher cost per partner. The board needs to define the commercial vs. scientific threshold — and who sets it for each engagement.

06 Scalability & Multi-Centre Strategy

⏱ 15 minutes

EXPANSION PHASES

PHASE 1 **0–12 Mo**	Validate model in one hospital · build replicable playbook · document all processes
PHASE 2 **12–24 Mo**	Replicate to 2–3 additional centres (same geography) · test playbook at scale
PHASE 3 **24–36 Mo**	Expand to 5–10 centres across geographies · begin international MedTech collaborations

REPLICABILITY CHALLENGES

- → Hospital buy-in varies significantly by institution culture and leadership
- → Data standards are non-uniform across Indian hospital systems
- → Clinical workflow cultures differ — one playbook may not fit all contexts
- → Regulatory and ethical requirements may vary by state and institution type

BOARD DISCUSSION

- ? Should the second centre be the same hospital system or a completely different one?
- ? Should partner hospitals become equity stakeholders in a network model?
- ? What is the funding model for multi-centre expansion?

07 Key Decisions & Next Steps

10 minutes

The following decisions are required before execution begins. Each needs a clear answer from this session.

DECISION 01

Founding Hospital Partner

Which hospital or health system will be the anchor? Is an MOU secured or pending?

DECISION 02

Revenue Model

Upfront fees, outcomes-based, or hybrid? What is the pricing for a validation engagement?

DECISION 03

Governance Structure

Founder-led initially or establish a formal steering committee from day one?

DECISION 04

Initial Partner Category

Focus on AI startups, MedTech, or enterprise first? Diagnostic AI as the beachhead?

DECISION 05

First AI Solution to Deploy

What clinical problem area — diagnostics, clinical workflow, or administrative AI? What is the selection criteria and who decides?

POST-BOARD ACTION ITEMS

ACTION	OWNER	DEADLINE	STATUS
Finalise hospital partnership MOU	Dr. Amit Raj	2 weeks	In Progress
Define and agree RWE validation framework	Anamika Singh	3 weeks	Pending Board Input
Identify first 3 AI solution candidates for pilot	Founding Team	3 weeks	Pending
Draft governance charter and steering committee structure	Dr. Amit Raj	4 weeks	Pending Board Input
Prepare technology infrastructure roadmap	Anamika Singh	4 weeks	Underway

Next Board Meeting

Timeline: 6–8 weeks from this session

Expected updates: Hospital partnership finalised · first pilot AI solutions identified · RWE framework approved · technology roadmap presented to board

Document prepared for Plexus AI Board of Advisors · April 2026 · Confidential